

Rowe, Boyd Patrick

MRN: 007087315

Office Visit 1/9/2025

Pav CC Head, Neck & Respiratory

Provider: Arnold, Susanne M, MD (Medical Oncology)

Primary diagnosis: Non-small cell lung cancer metastatic to brain (CMS/HCC)

Reason for Visit: Follow-up

Progress Notes

Yadav, Rina K, DO (Fellow) • Oncology

Cosigned by: Arnold, Susanne M, MD at 1/9/2025 3:40 PM

Attestation signed by Arnold, Susanne M, MD at 1/9/2025 3:40 PM (Updated)

I saw and evaluated the patient with the resident/fellow. I discussed the case with the resident/fellow and agree with the findings and plan as documented.

ADDENDUM 3:20pm

Subsequent to the above evaluation, the patient was undergoing an attempted thoracentesis, and had a vagal episode witnessed by Pulm medicine resident, with LOC and slumped over. The procedure was just beginning and he had lidocaine instilled into the chest wall, and was just beginning the procedure, (according to the physician performing the procedure, the pleural cavity was not yet accessed) when he had syncope, with posturing, LOC, clamminess, drop in BP, loss of control of bladder, and had a unilateral pupil enlargement (has baseline disconjugate gaze). He was placed in supine position, then became nauseous, and vomited, and then returned to supine position. Over 5 -7 minutes, he gradually had return of consciousness, and became conversant with stabilization of vital signs. At this point, given the severity of the reaction, he was transported for further monitoring to the ED.

MEDICAL ONCOLOGY FOLLOW-UP NOTE**Patient Information****Patient Name:** Boyd Patrick Rowe**MRN:** 007087315**Date of Birth:** 2/6/1980**REFERRING PHYSICIAN:**

Lawson, Terry A, APRN, CNS

Encounter Date: 1/9/2025**Treatment Diagnosis:**[Cancer Staging](#)

Adenocarcinoma of bronchus, left (CMS/HCC)

Staging form: Lung, AJCC 8th Edition

- Clinical stage from 12/2/2021: Stage IVB (cTX, cN3, pM1c) - Signed by Arnold, Susanne M, MD on 1/14/2022

History of Present Illness:Boyd Patrick Rowe is a 44 y.o. male who returns for followup of his [Cancer Staging](#)

Adenocarcinoma of bronchus, left (CMS/HCC), Staging form: Lung, AJCC 8th Edition, Clinical: Stage IVB. he

returns for follow up after recent hospitalization for COVID + acute hypoxic respiratory failure 2/2 new malignant pleural effusion that is PDL1+ (TPS 5-10%).

Oncology History Overview Note

His oncologic history is as follows:

1. A chest x-ray in early October 2021 was negative for pneumonia and for any kind of lung mass by report.
2. CT of the abdomen on October 21, 2021 showed lesions in the liver with a left lingular lung mass visualized by my review
3. Repeat CT scan without contrast on November 12, 2021 and with contrast on November 15th, 2021, revealed liver metastasis.
4. He was referred to Dr. Cavnar in early December for presumed metastasis, and this had a CT scan of the chest, on November 30th which revealed a lingular mass as well as bilateral lung nodules and mediastinal adenopathy as well as confirmation of liver metastasis and pelvic bony metastasis.
5. CT-guided biopsy on December 2, 2021, reveals an adenocarcinoma with a TPS score of 85% (PDL1 high).
6. MRI of the brain on December 7, 2021 shows 4 small brain metastasis (asymptomatic).
7. 1st cycle of pembrolizumab on 12/09/2021, but next generation sequencing reveals an EML4-ALK fusion. Pembrolizumab was stopped and he was switched to alectinib as of 1/20/22.
8. 1/20/22: Modest increase in size of 2 of 4 enhancing for index lesions. Remaining 2 are stable and no new lesions are identified. Chiari I malformation, with stable medullary compression at foramen of Monroe. Subsequent MRI shows decrease in size of brain mets.
9. 4/10/23: PET scan shows new intensely hypermetabolic R perihilar, supraclavicular, and upper abdominal lymph nodes concerning for progression of disease. Additional hypermetabolic activity of C7 vertebral body. Concern for pneumonitis.
10. 4/10/23: MRI brain shows no significant changes in the enhancing focus of the anterolateral R temporal lobe. Stable Chiari 1 malformation.

NGS Guardant 4-13-23		
Biomarker	Result	Actionable (Y/N)
ALK (EML4-ALK)	EML4-ALK fusion L1196Q	Y Resistance to alectinib and crizotinib
CDKN2A	R80*	

11. 4/20/23 presented with bilateral pulmonary emboli and started on Xarelto.
12. Began Lorlatinib 5/4/23.
13. S/p C7 Vertebral Body: 800 cGy in 1 fraction via 3D-CRT using 18X photons; delivered 05/30/2023
14. Hospitalized 5/27/24-12/30/24 for COVID + and AHRF 2/2 new malignant pleural effusion found to be PDL1+ (TPS 5-10%).

Adenocarcinoma of bronchus, left (CMS/HCC)

12/1/2021

Initial Diagnosis

Stage 4 malignant neoplasm of lung, right (CMS/HCC)

12/2/2021

Cancer Staged

Staging form: Lung, AJCC 8th Edition, Clinical stage from 12/2/2021: Stage IVB (cTX, cN3, pM1c) - Signed by Susanne M Arnold, MD on 1/14/2022

12/9/2021 -
12/9/2021

Chemotherapy

pembrolizumab (Keytruda) 400 mg in sodium chloride 0.9 % 100 mL chemo IVPB,
400 mg, Intravenous, Once, 1 of 24 cycles
Administration: 400 mg (12/9/2021)

Non-small cell lung cancer metastatic to bone (CMS/HCC)

5/16/2023

Initial Diagnosis

Non-small cell lung cancer metastatic to bone (CMS/HCC)

5/18/2023 -
8/30/2024

Radiation Therapy

The patient saw McGarry, Ronald C, MD for radiation treatment. This is the current list of radiation treatment:

SBRT: Right Spine - Cervical (Resolved)

	Treatment Period	Energy	Fraction Dose	Fraction s	Total Dose
Course C1	5/30/2023–5/30/2023 (days elapsed: 0)				
<i>Plans</i>	<i>Planned</i>				
CSpine	5/30/2023– 5/30/2023		800 cGy	1 / 1	800 cGy
<i>Reference Points</i>	<i>Delivered</i>				
CSpine	5/30/2023– 5/30/2023		—	—	800 cGy

Currently, he denies fever, or chills, but has continued sinus pressure, cough, sputum production, and is anxious as it has not gone away. He states that he shortness of breath has resolved since hospitalization and he is no longer wearing supplemental oxygen at night. States that he does get tachycardic with ambulation. All other 14 point ROS within normal limits.

Problem List and Medications

Reviewed and updated in this encounter by me personally

Objective

Performance Status

1: Restricted in physically strenuous activity but ambulatory and able to do light work
There were no vitals taken for this visit.

EXAM

Physical Exam

Vitals reviewed.

Constitutional:

General: He is not in acute distress.

Appearance: Normal appearance. He is not ill-appearing.

HENT:

Head: Normocephalic.

Right Ear: External ear normal.

Left Ear: External ear normal.

Nose: No congestion or rhinorrhea.
Comments: **Sinus tenderness**
Mouth/Throat:
Mouth: Mucous membranes are moist.
Pharynx: No oropharyngeal exudate (**mild thrush**).

Eyes:
Conjunctiva/sclera: Conjunctivae normal.
Pupils: Pupils are equal, round, and reactive to light.

Cardiovascular:
Rate and Rhythm: Regular rhythm. **Tachycardia** present.
Heart sounds: Normal heart sounds.

Pulmonary:
Effort: Pulmonary effort is normal.
Breath sounds: No wheezing, rhonchi or rales.

Musculoskeletal:
Cervical back: Normal range of motion. No tenderness.

Lymphadenopathy:
Cervical: No cervical adenopathy.

Neurological:
General: No focal deficit present.
Mental Status: He is alert and oriented to person, place, and time.
Motor: No weakness.

LABORATORIES STUDIES: reviewed by me personally today to monitor for cancer related drug toxicity and treatment related long term toxicity

CBC		
WBC 15.23	Hgb 14.5	PLT 403
	HCT 44.0	

Lab Results

Component	Value	Date
NEUTROABS	10.91 (H)	01/08/2025

BMPL			
Na 136	Cl 98	BUN 17	Gluc 109
K 4.0	Co2 24	Creat 0.97	

LIVER FUNCTION TESTING		
Tot Prot 6.5	AST 20	Tot bili 0.3
	ALT 17	Alkphos 85

Ca 9.2 | Mg 2.1 | Phos 3.5

Lab Results

Component	Value	Date
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RADIOLOGY:

I independently visualized the recent imaging below based on the patient's symptoms and oncologic history.

=== 12/16/24 ===

XR CHEST 2 VIEWS**FINDINGS:**

Moderate loculate left pleural effusion. Left lower lobe pulmonary opacities/collapse-consolidation. Cardiomegaly. Prominent bilateral hilar and pulmonary vasculature. No pneumothorax. Osseous structures are unremarkable.

- Impression -

Loculated small left pleural effusion. Recommend further evaluation with chest CT.

Final report signed by Conor M Lowry, MD on 12/17/2024 1:36 AM

Assessment/Plan **Cancer management:** [Cancer Staging](#)

Adenocarcinoma of bronchus, left (CMS/HCC)

Staging form: Lung, AJCC 8th Edition

- Clinical stage from 12/2/2021: Stage IVB (cTX, cN3, pM1c) - Signed by Arnold, Susanne M, MD on 1/14/2022
- He had progressed on Alectinib in lungs, bone, nodes, but with stable brain mets. Guardant 360 revealed a L1196Q mutation in addition to the previous EML4-ALK fusion, which makes his tumor resistant to further Alectinib or Crizotinib. Therefore switched to Lorlatinib on 5/4/23.
- I independently visualized the recent imaging given the patient's symptoms and oncologic history and discussed the current radiology findings with the patient in detail and answered all questions. I agree that this shows a small left effusion, of unclear significance in the setting of fever and infection.
- I previously reviewed liver and renal function as well as bone marrow function in relationship to this patient's ability to tolerate systemic cancer treatment
- He is fully disabled due to his metastatic lung cancer.
- Given evidence of disease progression with pleural effusion pathology consistent with disease progression, would recommended stopping Lorlatinib at this time.
- Will discuss patient's case at molecular tumor board to determine if patient should proceed with third line therapy (chemotherapy with carboplatin) versus Brigatinib which has been used as a 2nd line therapy option for ALK+ metastatic NSCLC per BrigALK2 study and Brigatinib in Patients With Crizotinib-Refractory Anaplastic Lymphoma Kinase-Positive Non-Small-Cell Lung Cancer: A Randomized, Multicenter Phase II Trial.
- Furthermore there have been case reports of novel ALK mutation with durable response to brigatinib after exposure to crizotinib and alectinib. However, not to Lorlatinib (Latif H, Liu SV. Novel ALK mutation with durable response to brigatinib-a case report. Transl Lung Cancer Res. 2020 Oct;9(5):2145-2148. doi: 10.21037/tlcr-20-145. PMID: 33209633; PMCID: PMC7653135.).

- Would likely benefit from a pleurex catheter to help with fluid re accumulation.
- MRI brain performed on 1/8/25 remains pending however personal review does not show evidence of disease.
- RTC after molecular tumor board.

2. PE

- Treated with Xarelto - plan for lifelong treatment - refilled

3. Treatment and lifestyle induced hyperlipidemia

- Due to persistent elevated lipids, continue Atorvastatin 80 mg, Fenofibrate and Evolocumab per Dr Arbune.
- Appreciate Dr. Arbune's opinion on management of hyperlipidemia

4. C7 bone met

- S/p palliative radiation

5. Hypothyroidism

- Continue Levothyroxine 50 mcg - TSH is WNL when last drawn in January
- Will need TSH level with next visit

6. Recurrent dysphagia (history of esophageal dilation)

- Refer to Dr. Fritz for eval, with CT neck and TSH prior

Discussed assessment and plan with attending, Dr. Arnold. Addendum per attending as listed above.

Rina K Yadav, DO
Hematology Oncology Fellow PGY6
330.1146

Orders Placed This Encounter

Procedures

- Ambulatory referral to Molecular Tumor Board
- Ambulatory referral to Pulmonology

Inactive Attestations

Removed attestation signed by Arnold, Susanne M, MD at 1/9/2025 3:40 PM (removed by Arnold, Susanne M, MD at 1/9/2025 3:40 PM)

I saw and evaluated the patient with the resident/fellow. I discussed the case with the resident/fellow and agree with the findings and plan as documented.

Progress Notes

Arnold, Susanne M, MD (Physician) • Oncology

MEDICAL ONCOLOGY FOLLOW-UP NOTE

Patient Information

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History of Present Illness:Boyd Patrick Rowe is a 44 y.o. male who returns for followup of his [Cancer Staging](#)

Adenocarcinoma of bronchus, left (CMS/HCC), Staging form: Lung, AJCC 8th Edition, Clinical: Stage IVB. he returns for followup and for reassessment of response to current therapy.

Oncology History Overview Note

His oncologic history is as follows:

1. A chest x-ray in early October 2021 was negative for pneumonia and for any kind of lung mass by report.
2. CT of the abdomen on October 21, 2021 showed lesions in the liver with a left lingular lung mass visualized by my review
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Biomarker	Result	Actionable (Y/N)
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CDKN2A	R80*	
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11. 4/20/23 presented with bilateral pulmonary emboli and started on Xarelto.
12. Began Lorlatinib 5/4/23.
13. S/p C7 Vertebral Body: 800 cGy in 1 fraction via 3D-CRT using 18X photons; delivered 05/30/2023
14. Hospitalized 12/27/24-12/30/24 for COVID + and AHRF 2/2 new malignant pleural effusion found to be PDL1+ (TPS 5-10%). COVID 19: positive; acute respiratory viral panel negative. He underwent thoracentesis at the bedside on 12/28. Was initially started on broad spectrum antibiotics, given no evidence of infection or sepsis those were stopped on D2. **COVID pneumonia found** on admission. Initially required O2 as above, and finished 3 days of remdesevir and was provided with dexamethasone to finish total 10 days on discharge.

Adenocarcinoma of bronchus, left (CMS/HCC)

12/1/2021	Initial Diagnosis Stage 4 malignant neoplasm of lung, right (CMS/HCC)
12/2/2021	Cancer Staged Staging form: Lung, AJCC 8th Edition, Clinical stage from 12/2/2021: Stage IVB (cTX, cN3, pM1c) - Signed by Susanne M Arnold, MD on 1/14/2022
12/9/2021 - 12/9/2021	Chemotherapy pembrolizumab (Keytruda) 400 mg in sodium chloride 0.9 % 100 mL chemo IVPB, 400 mg, Intravenous, Once, 1 of 24 cycles Administration: 400 mg (12/9/2021)

Non-small cell lung cancer metastatic to bone (CMS/HCC)

5/16/2023	Initial Diagnosis Non-small cell lung cancer metastatic to bone (CMS/HCC)
5/18/2023 - 8/30/2024	Radiation Therapy The patient saw McGarry, Ronald C, MD for radiation treatment. This is the current list of radiation treatment:

		Treatment Period	Energy	Fraction Dose	Fraction s	Total Dose
Course C1	Plans	5/30/2023–5/30/2023	(days elapsed: 0)			Planned
	CSpine	5/30/2023–5/30/2023		800 cGy	1 / 1	800 cGy
	Reference Points					Delivered
	CSpine	5/30/2023–5/30/2023		—	—	800 cGy

Currently, he denies fever, or chills, nausea, vomiting, dry mouth, throat pain, but has tachycardia, and improved shortness of breath, cough since the thoracentesis on 12/28/24. .

Pertinent records are reviewed today from recent hospitalization, COVID and thoracentesis.

Problem List and Medications

Reviewed and updated in this encounter by me personally

Objective

Performance Status

0: Fully active, able to carry on all pre-disease performance without restriction

Blood pressure (!) **152/110**, pulse 106, temperature 37 °C (98.6 °F), temperature source Oral, resp. rate 18, height 1.702 m (5' 7"), weight 101 kg (223 lb 5.2 oz), SpO2 94%.

EXAM

Physical Exam

Vitals reviewed.

Constitutional:

General: He is not in acute distress.

Appearance: Normal appearance. He is not ill-appearing.

HENT:

Head: Normocephalic.

Right Ear: External ear normal.

Left Ear: External ear normal.

Nose: **Congestion** and **rhinorrhea** present.

Comments: **Sinus tenderness**

Mouth/Throat:

Mouth: Mucous membranes are moist.

Pharynx: **Oropharyngeal exudate (mild thrush)** present.

Eyes:

Conjunctiva/sclera: Conjunctivae normal.

Pupils: Pupils are equal, round, and reactive to light.

Cardiovascular:

Rate and Rhythm: Normal rate and regular rhythm.

Heart sounds: Normal heart sounds.

Pulmonary:

Effort: Pulmonary effort is normal.

Breath sounds: Normal breath sounds. No wheezing, rhonchi or rales.

Musculoskeletal:

Cervical back: Normal range of motion. No tenderness.

Lymphadenopathy:

Cervical: No cervical adenopathy.

Neurological:

General: No focal deficit present.

Mental Status: He is alert and oriented to person, place, and time.

Motor: No weakness.

LABORATORIES STUDIES: reviewed by me personally today to monitor for cancer related drug toxicity and treatment related long term toxicity

CBC		
WBC 15.23	Hgb 14.5	PLT 403
	HCT 44.0	

Lab Results

Component	Value	Date
NEUTROABS	10.91 (H)	01/08/2025

BMPL			
Na 136	Cl 98	BUN 17	Gluc 109
K 4.0	Co2 24	Creat 0.97	

LIVER FUNCTION TESTING		
Tot Prot 6.5	AST 20	Tot bili 0.3
	ALT 17	Alkphos 85

Ca 9.2 | Mg 2.1 | Phos 3.5

Lab Results

Component	Value	Date
TSH	2.58	01/16/2024

RADIOLOGY:

I independently visualized the recent imaging below based on the patient's symptoms and oncologic history.

XR Chest 2 Views

Result Date: 12/17/2024

Impression: Loculated small left pleural effusion. Recommend further evaluation with chest CT. Final report signed by Conor M Lowry, MD on 12/17/2024 1:36 AM

=== 12/27/24 ===

CT CHEST ABDOMEN PELVIS W IV CONTRAST

Moderate left pleural effusion, slightly improved compared to prior most recent comparison but new from September 23, 2024, worrisome for pleural metastatic disease.

Slightly improved groundglass opacities in right upper lobe, consistent with infection or inflammation.

Similar intralobular septal thickening most likely reflecting edema.

No definite evidence of neoplastic progression.

Final report signed by Stephen B Hobbs, MD on 12/30/2024 3:37 PM

Assessment/Plan

44 y.o. year-old male who has a past medical history of Anxiety disorder, unspecified, GERD (gastroesophageal reflux disease), History of dysphagia, Hyperlipidemia, Hypertension, Hypothyroidism, Kidney stones, Lung cancer (on chemotherapy), and Obesity. who presents to UK Good Samaritan ED due shortness of breath with exertion.

Cancer management: [Cancer Staging](#)

Adenocarcinoma of bronchus, left (CMS/HCC)

Staging form: Lung, AJCC 8th Edition

- Clinical stage from 12/2/2021: Stage IVB (cTX, cN3, pM1c) - Signed by Arnold, Susanne M, MD on 1/14/2022
- S/p one dose of pembrolizumab and then noted to have an EML4-ALK fusion for lung cancer. HE was given Alectinib for 15 months and then progressed on Alectinib in lungs, bone, nodes, but with stable brain mets. Guardant 360 revealed a L1196Q mutation in addition to the previous EML4-ALK fusion, which makes his tumor resistant to further Alectinib or Crizotinib. Therefore switched to Lorlatinib on 5/4/23. He has now progressed in pleura last month and we will stop Lorlatinib and proceed with thoracentesis now, to hold him until we can start chemotherapy or he can get on clinical trial of 4th generation ALKi. Referral to Sarah Cannon Cancer Center made, since we don't have an ALK trial.
- I independently visualized the recent imaging given the patient's symptoms and oncologic history and discussed the current radiology findings with the patient in detail and answered all questions. I agree that this shows a very large left effusion, and no other evidence of new disease.
- I reviewed liver and renal function as well as bone marrow function in relationship to this patient's ability to tolerate systemic cancer treatment
- RTC in 2-3 weeks for review of repeated Guardant 360 blood test and for possible carbo-pemetrexed for salvage if he is not a candidate for Alki study.
- He is fully disabled due to his metastatic lung cancer.

2. Resolved Sepsis with acute hypoxic respiratory failure and COVID on 12/28/24

- COVID 19: positive; acute respiratory viral panel negative - received remdesivir
- I reviewed his CT PE results which showed no PE, in addition to large left pleural effusion, Interlobular septal thickening and GGO.
- Fluid analysis positive for adenocarcinoma
- discussed Pleurx drain with pt, and elected to perform thoracentesis today and repeat CXR in 2-3 weeks.
- Monitor pulse oxymetry

3.COVID pneumonia

COVID is positive on admission

Completed Remdesevir and dexamethasone

3. PE

- Treated with Xarelto - plan for lifelong treatment

4. Treatment and lifestyle induced hyperlipidemia

- Due to persistent elevated lipids, continue Atorvastatin 80 mg, Fenofibrate and Evolocumab per Dr Arbune.
- Appreciate Dr. Arbune's opinion on management of hyperlipidemia

5. C7 bone met

- S/p palliative radiation

6. Hypothyroidism

- Continue Levothyroxine 50 mcg - TSH is WNL when last drawn in January
- Will need TSH level with next visit

7. Recurrent dysphagia (history of esophageal dilation)

- Refer to Dr. Fritz for eval, with CT neck and TSH prior

ADDENDUM 3:20pm

Subsequent to the above evaluation, the patient was undergoing an attempted thoracentesis, and had a vagal episode witnessed by Pulm medicine resident, with LOC and slumped over. The procedure was just beginning and he had lidocaine instilled into the chest wall, and was just beginning the procedure, (according to the physician performing the procedure, the pleural cavity was not yet accessed) when he had syncope, with posturing, LOC, clamminess, drop in BP, loss of control of bladder, and had a unilateral pupil enlargement (has baseline disconjugate gaze). He was placed in supine position, then became nauseous, and vomited, and then returned to supine position. Over 5 -7 minutes, he gradually had return of consciousness, and became conversant with stabilization of vital signs. At this point, given the severity of the reaction, he was transported for further monitoring to the ED.

Susanne M Arnold, MD

Orders Placed This Encounter**Procedures**

- Ambulatory referral to Molecular Tumor Board
- Ambulatory referral to Pulmonology
- Ambulatory referral to Hematology/Medical Oncology

Instructions

After Visit Summary (Automatic SnapShot taken 1/9/2025)

Additional Documentation

Vitals: BP **152/110 !** (Abnormal) (BP Location: Left arm) Pulse 106 Temp 37 °C (98.6 °F) (Oral) Resp 18
Ht 1.702 m (5' 7") Wt 101 kg (223 lb 5.2 oz) SpO2 94% BMI 34.98 kg/m² BSA 2.19 m² Pain Sc 0-No pain

Flowsheets: Patient-Reported Data, Learning Needs Screening, Markey CARES Tobacco Treatment Program, Vital Signs, Vitals Reassessment

SmartForms: AMB SF STEADI FALL RISK

Communications

✉ Letter sent to Terry A Lawson, APRN, CNS

📎 Provider Notes

Media

From this encounter

Surprise Billing Notification - Electronic signature on 1/2/2025 9:58 AM - E-signed

🗣️ Mc Patient-Generated Subjective Global Assessment (Pg-Sga)

Question	1/2/2025 9:53 AM EST - Filed by Patient	9/16/2024 4:26 PM EST - Filed by Patient
Weight: In summary of my current and recent weight:		
Please select the type of measurement system you will be using for the next four questions regarding height and weight.	lbs/ft	lbs/ft
I currently weigh approximately:	220	215
My current height is approximately:	5 foot 7	5 foot 7 inches
One month ago I weighed approximately:	224	215
Six months ago I weighed approximately:	215	212
During the past two weeks my weight has:	not changed	not changed
Food intake		
As compared to my normal intake, I would rate my food intake during the past month as:	unchanged	unchanged
I am now taking	normal food but less than normal amount	
Symptoms: I have had the following problems that have kept me from eating enough during the past two weeks:		
No problems eating	No	No
No appetite, just did not feel like eating	No	No
Nausea	No	No
Constipation	No	No
Mouth sores	No	No
Things taste funny or have no taste	No	No
Problems swallowing	No	No
Pain	No	Yes
Where is your pain located?		Feet
Vomiting	No	No
Diarrhea	No	No
Dry mouth	No	No
Smells bother me	No	No
Feel full quickly	No	No
Fatigue	No	No
Other (Examples: depression, money, or dental problems)	No	No
Activities and Function		
Over the past month, I would generally rate my activity as:	not my normal self, but able to be up and about with fairly normal activities	not my normal self, but able to be up and about with fairly normal activities

🗣️ Welcome Accident Related Questionnaire

1/2/2025 9:57 AM EST -

Filed by Patient

Question

Is this visit related to a workers' compensation, auto, or Black Lung?

No

 Nccn Distress Questionnaire

Question	1/6/2025 8:47 AM EST - Filed by Patient	9/23/2024 5:39 PM EST - Filed by Patient
Appointment Type	Other	Surveillance appointment
Based on the past week, please select the number that best describes how much distress you have had. 0 being none and 10 being the most extreme.	3	4
Physical Concerns: Are you having difficulty with the following? Select: Yes or No		
Pain	No	Yes
Sleep	No	No
Fatigue	No	No
Tobacco use	No	No
Substance use	No	No
Memory/Concentration	No	No
Sexual health	No	No
Changes in eating	No	No
Loss or change of physical abilities	No	No
Emotional concerns: Are you having difficulty with the following? Select Yes or No		
Worry or anxiety	No	No
Sadness or Depression	No	No
Loss of interest or enjoyment	No	No
Grief or loss	No	No
Fear	No	No
Loneliness	No	No
Anger	No	No
Changes in appearance	No	No
Feelings of worthlessness or being a burden	No	No
Social Concerns: Are you having difficulties with the following? Select Yes or No		
Relationship with spouse or partner	No	No
Relationship with Children	No	No
Relationships with family members	No	No
Relationship with friends or coworkers	No	No
Communication with health care team	No	No
Ability to have children	No	No
Practical concerns: Are you having difficulties with following? Select Yes or No		
Taking care of myself	No	No
Taking care of others	No	No
Work	No	Yes
School	No	No
Housing	No	No
Finances	No	No
Insurance	No	No
Transportation	No	No

Question	1/6/2025 8:47 AM EST - Filed by Patient	9/23/2024 5:39 PM EST - Filed by Patient
Child care	No	No
Having enough food	No	No
Access to medicine	No	No
Treatment Decisions	No	No
Spiritual/Religious Concerns? Select Yes or No		
Sense of meaning or purpose	No	No
Change in faith or beliefs	No	No
Death, dying or afterlife	No	No
Conflict between beliefs and cancer treatments	No	No
Relationship with the sacred	No	No
Ritual or dietary needs	no	no
Other concerns		
Distress Total (range: 0 - 10)	3	4
Physical Concerns Total (range: 0 - 12)	0	0
Emotional Concerns Total (range: 0 - 9)	0	0
Social Concerns (range: 0 - 6)	0	0
Practical Concerns Total (range: 0 - 12)	0	0
Spiritual/Religious Concerns Total (range: 0 - 6)	0	0
NCCN Total (range: 0 - 42)	0	0

Education

No education to display

Linked Episodes

Oncology Treatment Noted 12/8/2021

Orders Placed

Ambulatory referral to Molecular Tumor Board Authorized

Ambulatory referral to Pulmonology Authorized

Ambulatory referral to Hematology/Medical Oncology Authorized

Medication Changes

As of 1/9/2025 3:39 PM

None

Medication List at End of Visit

As of 1/9/2025 3:39 PM

	Refills	Start Date	End Date
atorvastatin (Lipitor) 80 MG tablet	3	4/12/2024	4/12/2025
Take 1 tablet (80 mg) by mouth 1 (one) time each day. - Oral			
benzonatate (Tessalon Perles) 100 MG capsule	0	12/30/2024	—
Take 1 capsule (100 mg) by mouth 3 (three) times a day if needed for cough. Do not crush or chew. - Oral			
Evolocumab (Repatha SureClick) 140 MG/ML solution auto-injector	11	4/17/2024	4/17/2025
Inject 1 mL (140 mg) under the skin every 14 (fourteen) days. - Subcutaneous			
fenofibrate (Tricor) 48 MG tablet	11	5/28/2024	5/28/2025

	Refills	Start Date	End Date
Take 1 tablet (48 mg) by mouth 1 (one) time each day. - Oral			
guaifenesin (Mucinex) 600 MG 12 hr tablet	0	12/30/2024	1/9/2025
Take 1 tablet (600 mg) by mouth 2 (two) times a day if needed for cough for up to 10 days. Do not crush, chew, or split. - Oral			
hydrochlorothiazide (HYDRODiuril) 12.5 MG tablet	11	1/18/2024	—
Take 1 tablet (12.5 mg) by mouth 2 (two) times a day if needed (edema). - Oral			
ibuprofen 200 MG tablet	—		—
Take 4 tablets (800 mg) by mouth every 6 (six) hours if needed for mild pain. - Oral			
Patient-reported medication			
ipratropium-albuterol (Duo-Neb) 0.5-2.5 mg/3 mL nebulizer solution	11	12/30/2024	—
Take 3 mL by nebulization every 6 (six) hours if needed for wheezing or shortness of breath. - Nebulization			
levothyroxine (Synthroid, Levoxyl) 50 MCG tablet	2	12/9/2024	—
TAKE 1 TABLET BY MOUTH DAILY BEFORE BREAKFAST - Oral			
lorlatinib (Lorbrena) 100 MG tablet	3	1/8/2025	—
TAKE 1 TABLET BY MOUTH 1 TIME A DAY.TAKE WITH OR WITHOUT FOOD. DO NOT CRUSH, CHEW, OR DISSOLVE. SWALLOW TABLET WHOLE.			
montelukast (Singulair) 10 MG tablet	2	5/30/2024	2/24/2025
Take 1 tablet (10 mg) by mouth 1 (one) time each day. - Oral			
pantoprazole (Protonix) 40 MG EC tablet	2	9/26/2024	—
Take 1 tablet (40 mg) by mouth 1 (one) time each day before breakfast. - Oral			
rivaroxaban (Xarelto) 20 MG tablet	1	9/26/2024	—
Take 1 tablet (20 mg) by mouth 1 (one) time each day with dinner. Take with food. - Oral			

Visit Diagnoses

Primary: **Non-small cell lung cancer metastatic to brain (CMS/HCC)** C34.90, C79.31

COVID-19 U07.1

Anxiety disorder, unspecified type F41.9

Vagal reaction R55